



CELLWORKS GROUP, INC.

Response to HHS Request for Information

Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

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Submitted to: U.S. Department of Health and Human Services

From: Cellworks Group, Inc.

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RE: Comments on AI Adoption in Clinical Care – CLIA Laboratory Algorithmic Services

EXECUTIVE SUMMARY

Cellworks Group, a CLIA-certified precision oncology laboratory, submits these comments to assist HHS in accelerating responsible AI adoption in clinical care. We focus on a critical barrier: algorithmic services offered lawfully by CLIA-certified laboratories currently lack clear pathways to obtain Medicare coverage, codes, and payment rates on the same terms as other laboratory services.

We offer three specific recommendations addressing the RFI's focus areas:

Regulation: Clarify that CLIA oversight combined with rigorous technical assessment provides sufficient regulatory framework for single-site algorithmic laboratory services, enabling Medicare coverage without requiring FDA clearance as a prerequisite, while supporting public trust in diagnostic outputs.

Reimbursement: Endorse the AMA's Clinically Meaningful Algorithmic Analyses (CMAA) framework to provide predictable coding and payment for algorithmic services, with payment rates reflecting the substantial resources required to develop and validate these services.

Research & Development: Fund multi-institutional outcomes research to generate real-world evidence supporting payer coverage decisions for validated algorithmic services, with emphasis on health equity and access for all Americans.

These recommendations directly address HHS's stated goals: reducing uncertainty that impedes AI innovation, fostering public trust and confidence in AI tools, aligning federal incentives, and accelerating access to validated AI tools in clinical care for patients nationwide.



ABOUT CELLWORKS

Cellworks Group operates a CLIA-certified laboratory developing algorithmic analyses that integrate genomic data with treatment simulation modeling for precision oncology. Our services:

- Undergo rigorous analytical validation under CLIA standards
- Complete rigorous technical assessment through established review processes (e.g., MolDx) that evaluate clinical utility
- Generate molecular guidance that physicians incorporate alongside other clinical factors in therapy decision-making
- Are validated against NCCN clinical guidelines and real-world treatment outcomes

We have direct experience with the regulatory and reimbursement barriers this RFI addresses.

REGULATION: CLARIFY CLIA OVERSIGHT FOR LABORATORY ALGORITHMIC SERVICES

The Regulatory Barrier

Stakeholders face uncertainty about whether algorithmic services require FDA clearance to be eligible for Medicare coverage. This uncertainty particularly affects CLIA-certified laboratory services that:

- Are performed exclusively within a single laboratory facility
- Integrate with core laboratory testing (genomic sequencing, molecular analysis)
- Operate under continuous CLIA regulatory oversight
- Are issued under laboratory director responsibility

The High Bar CLIA Services Already Meet

CLIA-certified laboratories offering algorithmic services undergo rigorous oversight:

Analytical Validation: Sensitivity, specificity, reproducibility, bias evaluation

Personnel Standards: Director qualifications, competency assessments, continuing education

Quality Assurance: Proficiency testing, internal quality controls, performance monitoring



Technical Assessment: Many services undergo additional voluntary rigorous technical assessment (e.g., MoIDx, NYSDOH, or equivalent) that evaluates clinical utility and evidence standards - a high bar that ensures patient benefit

CLIA's personnel qualifications, proficiency testing, and quality assurance requirements provide a durable governance foundation that supports patient and clinician trust in diagnostic outputs.

This oversight framework has successfully supported major clinical diagnostics markets.

Example: Oncotype DX, a widely adopted algorithmic diagnostic for breast cancer treatment prediction, was developed as CLIA-certified LDT with extensive prospective validation. Today it:

- Informs ~40% of U.S. breast cancer treatment decisions (>90% in relevant subtypes)
- Is Medicare/Medicaid reimbursed nationwide
- Is recommended in NCCN clinical guidelines
- Has 20+ years of prospective outcomes data

Recommendation

HHS should clarify that CLIA oversight combined with rigorous technical assessment provides sufficient regulatory framework for single-site laboratory algorithmic services to be eligible for Medicare coverage and payment. This clarification would:

- Remove unnecessary regulatory uncertainty blocking commercialization
- Recognize that CLIA + rigorous technical assessment establishes rigorous safeguards
- Enable Medicare coverage decisions based on clinical evidence and utility
- Accelerate access to validated algorithmic services
- Foster public confidence in AI tools through transparent governance

Implementation: CMS should issue guidance stating that CLIA-certified laboratory algorithmic services meeting established rigorous technical assessment standards (e.g., MoIDx, NYSDOH or equivalent) are eligible for Medicare coverage consideration without FDA clearance as a prerequisite.

REIMBURSEMENT: ENABLE PREDICTABLE CODING AND ADEQUATE PAYMENT

The Reimbursement Barrier

The current coding and payment landscape creates two critical barriers for algorithmic laboratory services:

1. Coding Uncertainty



- Existing CPT structures (particularly PLA codes) create laboratory-specific codes rather than function-specific codes
- This prevents multiple validated laboratories from competing on the same code
- Results in 2-3+ year delays as each laboratory seeks unique codes and payer coverage

2. Payment Inadequacy

- Current payment methodologies may not reflect the substantial resources required to:
 - Develop and validate algorithmic services
 - Conduct prospective outcomes studies
 - Maintain ongoing quality surveillance
 - Update algorithms based on emerging evidence

The Solution: CMAA Framework

The AMA CPT Panel is developing a Clinically Meaningful Algorithmic Analyses (CMAA) framework that would:

- Create function-specific (not lab-specific) codes for algorithmic services
- Include a transition pathway for existing PLA-coded services to migrate to function specific categories.
- Organize codes by clinical evidence depth and utility
- Enable multiple laboratories meeting evidence standards to use the same codes
- Support competitive markets based on quality and outcomes

This framework directly solves the coding bottleneck HHS has identified.

Recommendation

HHS should publicly endorse the AMA CMAA framework and commit to implementation:

- 1 CMS Endorsement: Issue a statement supporting CMAA as the appropriate coding structure for clinical algorithmic services
- 2 Payment Commitment: Commit to Medicare payment implementation by January 1, 2027, with:



- Payment rates under Clinical Laboratory Fee Schedule (CLFS) for laboratory-based algorithmic services
- Rates reflecting the substantial validation and development burden
- Methodology accounting for ongoing surveillance and evidence generation

3 Multi-Payer Alignment: Encourage Medicaid and commercial payers to adopt CMAA coding to create consistent national coverage framework

Implementation: CMS should coordinate with AMA CPT Panel to ensure CMAA codes are Medicare-payable upon finalization, and that CLFS payment methodology appropriately values algorithmic laboratory services.

RESEARCH & DEVELOPMENT: FUND OUTCOMES INFRASTRUCTURE

The Evidence Gap

While many algorithmic services demonstrate strong analytical and clinical validity, payers increasingly require real-world outcomes evidence before providing coverage. However:

- Single laboratories cannot fund multi-institutional prospective studies
- Academic medical centers lack incentives to generate comparative effectiveness data
- Payers wait for outcomes evidence before covering services (creating a catch-22)

Recommendation

HHS should establish an "Algorithmic Diagnostics Validation Initiative" to generate outcomes evidence:

Structure:

- \$50-100M annual funding primarily through AHRQ, with NIH coordination
- Competitive grants for multi-institutional outcomes studies
- Focus on algorithmic services with strong preliminary evidence but limited real-world validation
- Require data sharing to support multiple payers' coverage decisions

Priorities:

- Prospective outcomes studies comparing algorithmic-guided vs. standard-of-care treatment selection
- Health economics analyses (cost-effectiveness, value-based care implications)



- Implementation science (workflow integration, physician adoption barriers)
- Health equity studies ensuring algorithm performance across diverse populations. When appropriately validated and monitored, algorithmic laboratory services can help democratize access to expert-level oncology analysis for patients regardless of geography - supporting equitable care for individuals treated in rural or community settings as well as those at academic centers.

Expected Impact:

- Accelerates payer coverage decisions by generating required evidence
- Reduces commercial barriers for validated algorithmic services
- Creates public evidence infrastructure supporting competitive markets
- Identifies high-value algorithmic applications for federal investment
- Advances health equity by ensuring AI tools serve all Americans effectively

ADDRESSING RFI PRIORITY QUESTIONS

What are the biggest barriers to AI adoption in clinical care?

For CLIA laboratory algorithmic services:

- 1 Regulatory uncertainty: Lack of clarity on whether CLIA oversight combined with rigorous technical assessment is sufficient for Medicare eligibility
- 2 Coding bottleneck: PLA code structure prevents function-specific coding and delays market entry
- 3 Payment inadequacy: Current methodologies may undervalue development and validation burden
- 4 Evidence gap: Payers require outcomes evidence that single labs cannot generate

What regulatory or payment policy changes should HHS prioritize?

- 1 Immediate (2026): CMS guidance clarifying CLIA + rigorous technical assessment (e.g. MoIDx) sufficiency for Medicare coverage eligibility
- 2 Near-term (2027): CMAA framework implementation with committed Medicare payment under CLFS
- 3 Medium-term (2027-2028): Outcomes research funding to generate coverage-decision evidence



Where have AI tools met or exceeded expectations?

Oncotype DX demonstrates that algorithmic diagnostics developed as CLIA-certified LDT:

- Standard-of-care adoption (>90% utilization in relevant patient populations)
- Guideline recommendation (NCCN, ASCO)
- Nationwide payer coverage (Medicare, Medicaid, commercial)
- 20+ years sustained clinical use with continuous outcomes validation

This precedent proves that CLIA-governed laboratory services with rigorous validation can support major clinical markets when evidence standards are high.

CONCLUSION

Algorithmic services offered lawfully by CLIA-certified laboratories face unnecessary barriers to obtaining Medicare coverage, codes, and payment on the same terms as other laboratory services. These barriers throttle AI adoption despite strong clinical evidence and patient benefit.

HHS can address these barriers through three coordinated actions:

- 1 Clarify that CLIA oversight + rigorous technical assessment (e.g. MoLDx) provides sufficient regulatory framework for Medicare coverage consideration
- 2 Endorse CMAA coding framework with committed Medicare payment implementation
- 3 Fund outcomes research infrastructure to generate coverage-decision evidence and advance health equity

These actions align with HHS's stated goals: reducing regulatory uncertainty, fostering public trust in AI tools, enabling innovation, and accelerating access to validated AI services that improve outcomes for all Americans.

Cellworks remains available to provide technical input, clinical perspective, or implementation experience as HHS refines policies to accelerate responsible AI adoption in clinical care.



APPENDIX: SUPPORTING EVIDENCE

CLIA + Rigorous Technical Assessment as Comprehensive Safeguard

Rigorous technical assessment processes (e.g., MoIDx) require:

- Analytical Validity: Evidence of accuracy, reproducibility, and performance across populations
- Clinical Validity: Literature demonstrating correlation with clinical outcomes
- Clinical Utility: Evidence that patients benefit when results guide decision-making
- Post-Market Surveillance: Ongoing performance monitoring and outcomes tracking

This framework has successfully validated 200+ laboratory tests with demonstrated clinical adoption, establishing that rigorous technical assessment combined with CLIA oversight creates a high bar for quality and patient safety.

Economic Impact of Reimbursement Barriers

Current barriers create:

- 2-3 year delays in payer coverage per algorithmic service
- \$5-10M per service in duplicative evidence generation (each payer requires separate dossiers)
- Venture funding hesitation due to reimbursement uncertainty
- Innovation suppression as companies avoid algorithmic development

CMAA implementation would:

- Reduce coverage timeline from 2-3 years to 12-18 months
- Create predictable coding structure enabling commercial planning
- Support competitive markets (multiple labs using same codes)
- Accelerate innovation by removing reimbursement uncertainty

